APEXCARE INDIA EMPLOYEE HEALTH BENEFIT PROGRAMME (AIEHBP)
MASTER GROUP HEALTH POLICY WORDING

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Primary audience: Members, HR, insurer, TPA and claims teams

FICTIONAL TRAINING DOCUMENT - NOT AN INSURANCE CONTRACT. This document is a synthetic example created for software development, retrieval testing, and claims-workflow demonstrations. A real insurer, employer, TPA, legal team, medical team, and compliance function must review and approve all wording, benefits, timelines, forms, and regulatory references before operational use.

Document-control rule: cite the document code, section, clause, page, and version in every decision.
Interpretation rule: the signed policy schedule and endorsements override this demonstration text.

1. PURPOSE, CONTRACT STRUCTURE AND PRECEDENCE

This master wording defines the common coverage rules for the fictional Corporate Standard Plan. It must be read with the benefit schedule, endorsements and member certificate.

POL-1.1 Policy components

The contract set consists of the signed policy schedule, member certificate, endorsements, this master wording, the benefit schedule and accepted proposal information. Operational guides explain procedures but cannot enlarge or reduce cover.

POL-1.2 Order of precedence

A signed endorsement prevails over the policy schedule only for the subject it amends. The policy schedule prevails over this wording. This wording prevails over member guides and internal procedures. A later effective version prevails only for claims occurring during that version period.

POL-1.3 Evidence-based interpretation

No benefit is payable merely because a treatment is medically useful. Payment requires eligibility, an active cover period, an insured benefit, medical necessity, required evidence, and absence of an applicable exclusion or limit.

POL-1.4 No silent assumptions

If member identity, plan, effective date, diagnosis, procedure, provider, or policy evidence is uncertain, the claim must be marked PENDING_INFORMATION or MANUAL_REVIEW rather than accepted or rejected by inference.

2. ELIGIBILITY, ENROLLMENT AND COVER PERIOD

Coverage depends on validated enrollment and dates.

POL-2.1 Eligible employees

A permanent employee listed by the policyholder and accepted in the enrollment file is eligible from the effective date communicated in the member certificate.

POL-2.2 Eligible dependants

Eligible dependants may include one lawful spouse or declared partner, dependent children from birth through age twenty-five while meeting dependency rules, and parents or parents-in-law only when the schedule expressly includes them.

POL-2.3 New joiners and newborns

New employees and newly eligible dependants must be enrolled within the employer notification window. A newborn may receive cover from birth when maternity/newborn cover is active and notification is received within thirty days.

POL-2.4 Termination

Cover ends on the earliest of policy expiry, termination recorded by the policyholder, exhaustion where restoration does not apply, or another date shown in an endorsement. Treatment after termination is not covered merely because symptoms began earlier.

POL-2.5 Identity mismatch

Minor spelling differences may be resolved using government identity, employee records and hospital records. Material mismatch must be investigated; mismatch alone is not proof of fraud.

3. CORE HOSPITALIZATION COVER

This section defines the principal insured event.

POL-3.1 In-patient hospitalization
Reasonable and customary expenses are eligible when the member is admitted for at least twenty-four consecutive hours for medically necessary treatment on written advice of a registered medical practitioner.

POL-3.2 Day-care procedures
The twenty-four-hour requirement does not apply to listed or clinically established day-care procedures requiring hospital infrastructure because technological advancement permits earlier discharge.

POL-3.3 Covered expense categories
Subject to the schedule, eligible costs include room, nursing, ICU, surgeon, anaesthetist, physician, diagnostic tests, medicines, implants, operation theatre and medically necessary consumables that are not expressly non-payable.

POL-3.4 Medical necessity
The diagnosis, setting, duration and intensity of care must be consistent with generally accepted clinical practice. Experimental convenience or provider preference alone does not establish medical necessity.

POL-3.5 Reasonable and customary charge
The payable charge may be limited to the usual charge for comparable treatment, provider class and locality. Any reduction must identify the benchmark and affected line item.

POL-3.6 Emergency stabilization
Emergency stabilization is covered when delay would reasonably threaten life, limb or organ function. Network authorization requirements may be relaxed, but notification and evidence requirements still apply as soon as practicable.

4. ASSOCIATED BENEFITS

These benefits apply only when shown as active in AIEHBP-BEN-002.

POL-4.1 Pre-hospitalization

Medical expenses incurred during the thirty days immediately before an admissible hospitalization are eligible when directly related to the same condition.

POL-4.2 Post-hospitalization

Medical expenses incurred during the sixty days immediately after discharge are eligible when directly related to the same condition and supported by prescriptions and invoices.

POL-4.3 Road ambulance

Emergency road ambulance charges to the nearest suitable hospital are payable up to the schedule limit when medically necessary.

POL-4.4 AYUSH hospitalization

In-patient treatment under Ayurveda, Yoga and Naturopathy, Unani, Siddha or Homeopathy is eligible only at a recognized facility and subject to the same necessity and documentation standards.

POL-4.5 Organ donor expenses

Hospitalization expenses of an organ donor may be eligible when the recipient is an insured member and the transplant claim is admissible. Organ acquisition, trafficking, donor screening unrelated to an approved transplant and donor follow-up are not covered unless listed.

POL-4.6 Domiciliary treatment

Home treatment is eligible only when hospitalization would otherwise be required but transfer is impossible or hospital accommodation is unavailable, and daily clinical records are supplied.

5. MATERNITY, NEWBORN AND SPECIAL BENEFITS

Special benefits have independent limits and conditions.

POL-5.1 Maternity

Hospital expenses for childbirth, including medically necessary caesarean delivery, are covered only when maternity is active and the member satisfies the waiting period or an endorsement waives it.

POL-5.2 Newborn care

A newborn is covered from birth up to the maternity/newborn sublimit for medically necessary care when timely enrollment is completed. Congenital conditions are assessed under the schedule and applicable law, not automatically excluded.

POL-5.3 Infertility and assisted reproduction

Investigation or treatment of infertility, sterilisation reversal and assisted reproductive technology are excluded unless a signed endorsement expressly includes them.

POL-5.4 Mental healthcare

Medically necessary mental healthcare is assessed under the same core principles and any schedule-specific limits. A diagnosis must not be rejected solely because it is psychiatric.

POL-5.5 Modern treatment methods

Listed modern treatment methods are eligible subject to medical necessity, schedule sublimits and applicable authorization. The method must not be rejected merely because it uses newer technology.

6. LIMITS, COST SHARING AND CALCULATION

Benefits are calculated after eligibility and exclusion review.

POL-6.1 Sum insured

The family floater sum insured is shared by enrolled family members. The available balance is measured on the admission date and adjusted for earlier accepted claims.

POL-6.2 Room eligibility

Room and ICU eligibility follow the schedule. Where a proportionate deduction applies, it must be calculated only according to the schedule and must exclude items that do not vary with room category.

POL-6.3 Deductible

A deductible is the eligible amount borne by the member before insurer payment. It is not applied when the schedule states NIL.

POL-6.4 Co-payment

Co-payment is the percentage of an otherwise admissible amount borne by the member. It is applied after non-payable items, sublimits and deductible unless an endorsement states otherwise.

POL-6.5 Restoration

Restoration replenishes the sum insured only under the trigger and reuse conditions in the schedule. It cannot increase a benefit-specific sublimit.

POL-6.6 Other insurance

When another policy covers the same event, coordination must follow applicable contribution rules and the member may select a primary policy where permitted. Original document handling must be recorded.

7. CLAIMS DUTIES AND DECISION STATES

Procedural duties support timely assessment but must be applied fairly.

POL-7.1 Notification

Planned admissions require preauthorization before admission. Emergency admissions should be notified within twenty-four hours or as soon as reasonably possible.

POL-7.2 Reimbursement submission

A reimbursement claim should be submitted within thirty days after discharge unless the schedule or law provides a different period. Delay supported by reasonable cause must be reviewed rather than automatically rejected.

POL-7.3 Cooperation

The member must provide accurate information, authorization to obtain relevant records and reasonable assistance with verification.

POL-7.4 Decision states

Permitted workflow states are RECEIVED, PENDING_INFORMATION, UNDER_MEDICAL_REVIEW, APPROVED, PARTIALLY_APPROVED, REJECTED, WITHDRAWN and MANUAL_REVIEW.

POL-7.5 Reasoned decision

Every reduction or rejection must cite the controlling clause, explain facts relied upon, show the calculation where relevant, and describe review or grievance options.

8. GENERAL INTERPRETATION AND SAFEGUARDS

These safeguards apply across all claims.

POL-8.1 Conflict or ambiguity

Conflicting clauses, missing endorsements or uncertain scope require manual review. The system must not select whichever clause produces the lowest payment.

POL-8.2 Fraud allegation

A discrepancy or red flag triggers verification; it does not by itself prove fraud. An adverse fraud decision requires documented evidence and authorized review.

POL-8.3 Clinical judgement

Automated extraction or similarity scores cannot replace required medical review for necessity, causation, coding, experimental status or complex comorbidity.

POL-8.4 Human oversight

No generative model may issue a final adverse decision without the controls, evidence and approval authority defined in AIEHBP-SOP-006.

9. DEFINITIONS

Defined terms should be attached to relevant chunks during retrieval.

POL-9.1 Accident
A sudden, unforeseen and involuntary event caused by external, visible and violent means.

POL-9.2 Hospital
An institution legally registered to provide in-patient care with qualified nursing and medical supervision, records, beds and required facilities.

POL-9.3 Illness
A sickness or disease first manifesting or requiring treatment during the coverage period, subject to waiting-period rules.

POL-9.4 Pre-existing disease
A condition diagnosed, treated, advised, or showing relevant signs or symptoms before the first continuous coverage date, assessed using applicable policy and regulatory definitions.

POL-9.5 Registered medical practitioner
A person validly registered with the appropriate medical council and acting within the lawful scope of practice.

POL-9.6 Network provider
A provider contracted for cashless service for the relevant policy and date; network status does not guarantee that every treatment is covered.